

LCD Reference Article Billing and Coding Article

Billing and Coding: Cognitive Assessment and Care Plan Service

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Palmetto GBA	A and B MAC	10111 - MAC A	J - J	Alabama
Palmetto GBA	A and B MAC	10112 - MAC B	J - J	Alabama
Palmetto GBA	A and B MAC	10211 - MAC A	J - J	Georgia
Palmetto GBA	A and B MAC	10212 - MAC B	J - J	Georgia
Palmetto GBA	A and B MAC	10311 - MAC A	J - J	Tennessee
Palmetto GBA	A and B MAC	10312 - MAC B	J - J	Tennessee

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Palmetto GBA	A and B and HHH MAC	11201 - MAC A	J - M	South Carolina
Palmetto GBA	A and B and HHH MAC	11202 - MAC B	J - M	South Carolina
Palmetto GBA	A and B and HHH MAC	11301 - MAC A	J - M	Virginia
Palmetto GBA	A and B and HHH MAC	11302 - MAC B	J - M	Virginia
Palmetto GBA	A and B and HHH MAC	11401 - MAC A	J - M	West Virginia
Palmetto GBA	A and B and HHH MAC	11402 - MAC B	J - M	West Virginia
Palmetto GBA	A and B and HHH MAC	11501 - MAC A	J - M	North Carolina

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Palmetto GBA	A and B and HHH MAC	11502 - MAC B	J - M	North Carolina

Article Information

General Information

Article ID

A59036

Article Title

Billing and Coding: Cognitive Assessment and Care Plan Service

Article Type

Billing and Coding

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CMS National Coverage Policy

Title XVIII of the Social Security Act, §1833(e) prohibits Medicare payment for any claim which lacks the necessary information to process the claim.

CMS Internet-Only Manual, Pub. 100-02, Medicare Benefit Policy Manual, Chapter 15, §280.5.1 Advance Care Planning (ACP) Furnished as an Optional Element with an Annual Wellness Visit (AWV) Upon Agreement with the Patient.

Federal Register, CMS 1654-F, Vol. 81, No. 220, November 15, 2016, p. 80252 addresses assessment and care planning for patients with cognitive impairment (GPPP6).

Federal Register, CMS 1676-F, Vol. 82, No. 219, November 15, 2017, p. 53077 addresses assessment of and care planning for patients with cognitive impairment (CPT code 99483).

Federal Register, CMS 1734-F, Vol. 85, No. 248, December 28, 2020, p. 84749 addresses assessment of and care planning for patients with cognitive impairment (CPT code 99483).

Medicare Learning Network. CMS. MLN5343505 March 2021. Cognitive Assessment & Care Plan Services CPT Code 99483.

Medicare Learning Network. MLNConnects. July 15, 2021. Alzheimer's Impact Movement™. FACTSHEET, CPT® Code 99483 Explanatory Guide for Clinicians-March 2020.

Centers for Medicare and Medicaid Services. Cognitive Assessment and Care Plan Services. (For Health Care providers).

Article Guidance

Article Text

The information in this article contains billing, coding or other guidelines that complement the Local Coverage Determination (LCD) for Cognitive Assessment and Care Plan Service L39266.

Code 99483 provides reimbursement to physicians and other eligible billing practitioners for a comprehensive clinical visit that results in a written care plan.

All beneficiaries who are cognitively impaired are eligible to receive the services under this code. This includes those who have been diagnosed with Alzheimer's, other dementias, or mild cognitive impairment. It also includes those individuals without a clinical diagnosis who, in the judgment of the clinician, are cognitively impaired.

Screening for cognitive impairment is still a requirement of Medicare Annual Wellness Visits (AWV). However, cognitive impairment can also be identified as part of a routine visit through direct observation or by considering information from the patient, family, friends, caregivers, and others. You may perform a more detailed cognitive assessment and develop a care plan during a separate visit.

CPT code 99483 is generally billed separately from the annual wellness visit due to the time, complexity and medical decision making inherent to this service. However, if the AWWV and Cognitive Assessment and Care plan services are done at the same visit, a -25 modifier would need to be appropriately utilized.

Code 99483 requires an independent historian in order to correctly perform the assessments and develop a corresponding care plan under CPT code 99483. An independent historian can be a parent, spouse, guardian, or any other individual who can provide patient history when a patient is not able to supply complete or reliable information.

Required service elements for CPT code 99483 include **ALL** of the following:

- Cognition-focused evaluation, including a pertinent history and examination;
- Medical decision making of moderate or high complexity;
- Functional assessment (e.g., Basic and Instrumental Activities of Daily Living), including decision-making capacity;
- Use of standardized instruments to stage dementia (e.g., Functional Assessment Staging Test [FAST], Clinical Dementia Rating [CDR]);
- Medication reconciliation and review for high-risk medications;
- Evaluation for neuropsychiatric and behavioral symptoms, including depression and including use of standardized instruments;
- Evaluation of safety, at home and otherwise, including motor vehicle operation, if applicable;
- Identification of caregiver(s), caregiver knowledge, caregiver needs, social supports and willingness of caregiver to take on caregiving tasks;
- Development, with periodic updating/revision/review of an Advance Care Plan;
- Creation of a written care plan which includes initial plans to address any neuropsychiatric symptoms, neurocognitive symptoms, functional limitations, and referral to community resources as needed. This care plan must be documented as having been shared with the patient and/or caregiver at the time of initial education and support.

The components noted above are central to informing, designing and delivering a care plan suitable for patients with cognitive impairment.

Typically, 50 minutes are spent face-to-face with the patient and/or family or caregiver for this service.

Do not report cognitive assessment and care plan services if any of the required service elements are not performed or are deemed unnecessary for the patient's condition. For these services, see the appropriate evaluation and management (E/M) code.

DOCUMENTATION

The documentation of cognitive-relevant history should include any factors that could be contributing to cognitive impairment such as, but not limited to, psychoactive medications, chronic pain syndromes, infection, depression, and other brain diseases.

The documentation of medical decision making should include current and likely progression of the patient's disease, the need for referral(s) for rehabilitative, social, legal, financial, or community services including meal/transportation and other personal assistive services.

Identification of patients eligible for this service for whom no diagnosis has yet been firmly established will require excellent documentation that confirms the presence of cognitive impairment and provides the narrative history that spurred suspicion for a potential cognitive impairment diagnosis on the part of the practitioner.

Standardized, validated tools should be used and are required for some elements. Such tools offer a basic framework on which to build a nuanced clinical understanding of care needs through ongoing clinical contact with the patient and caregiver. Though all required elements must be represented, the choice of assessment tools should be customized for differing clinician styles and practice composition, workflows and overall clinical goals. However, all assessment tools utilized must be able to demonstrate standardization, validation and be recognized as such by reputable national specialty organizations. Palmetto GBA reserves the right to review the accuracy, reliability, efficacy, and general credibility of assessment measurement tools utilized. Specific documentation requirements using various standardized measurement tools are noted below. These requirements of course do not preclude the use of additional assessment methods as desired by the individual practitioners.

For any scoring tool assessment performed, the full instrument raw scoring and results must be available for Medicare Administrative Contractor review if requested.

Documentation of cognitive assessment must be present within the medical record. Alzheimer's Association suggestions for cognitive measurement tools are being adopted by Palmetto GBA as acceptable and reasonable/necessary methods. Thus, assessment of cognition via the Mini-Cog[®] or GPCOG or Short Montreal Cognitive Assessment (s-MoCA) tools must be documented within the medical record.

A functional assessment of basic and instrumental activities of daily living with either the Katz Index of Independence in Activities of Daily Living or the Lawton-Brody Instrumental Activities of Daily Living Scale (IADL) is required and must be present within the medical record. Use of a standardized instrument for the staging of dementia – either the Functional Assessment Staging Test (FAST scale), Clinical Dementia Rating (CDR[®] Dementia Staging Instrument), Dementia Severity Rating Scale (DSRS), or Global Deterioration Score (GDS) is required and again must be present within the medical record. These dementia staging tools require the involvement of an independent historian. This historian must be identified within the medical record as having been present and involved.

Decision-making capacity of the patient must be documented within the medical record. Various methods/tools may be used for this assessment. Essentially the decision-making capacity of the patient is based on global clinician judgment. The practitioner should at least note if the patient is able to make their own decisions or is not able to make their own decisions or that decision making capacity is uncertain and will require further evaluation. Final care planning must result in identification of who will be making decisions in the event that the patient cannot.

Because the identification of co-existing neuropsychiatric symptoms or conditions is so important and because this assessment with standardized measure tools is required for the 99483 CPT service, the following Alzheimer's Association supported tools will be acceptable for purposes of coverage: Neuropsychiatric Inventory Questionnaire (NPI-Q) or BEHAV5+[®] or Patient Health Questionnaire-2 (PHQ-2). Evidence of use of 1 of these assessment methods must be present within the medical record.

Documentation demonstrating full reconciliation of medications must be present.

Evaluation of safety for home and motor vehicle operation must be documented.

Social supports must be identified and documented. The documentation must be clear as to how much caregivers know about the patient and how much care they are willing to provide. Caregivers should be identified within the record.

Advance Care Planning must be addressed as well as any palliative care needs.

In general, the Alzheimer's Association's care planning toolkit is a comprehensive resource that many practitioners will find very helpful. [Cognitive Impairment Care Planning Toolkit \(alz.org\)](https://www.alz.org/careplanning-toolkit) 

The written care plan should reflect a synthesis of the information acquired as part of the assessment. It should be written in language that is easily understood, indicates who has responsibility for carrying out each recommended action step and specify an initial follow-up schedule.

PROVISION OF THE COGNITIVE ASSESSMENT/PLANNING SERVICE

Any practitioner eligible to report E/M services can provide this service. Eligible providers include physicians (MD and DO), nurse practitioners (NP), clinical nurse specialists (CNS), certified nurse midwives (CNM), and physician assistants (PA). Eligible practitioners must provide documentation that supports a moderate-to-high level of complexity in medical decision making, as defined by E/M guidelines (with application as appropriate of the usual "incident-to" rules, consistent with other E/M services).

The 10 assessment elements of 99483 can be evaluated within the care planning visit or in 1 or more visits that precede it, using appropriate billing codes (most often an E/M code). Patients with complex medical, behavioral, psychosocial and/or caregiving needs may require a series of assessment visits, while those with well-defined or less complex problems may be fully assessed during the care plan visit. Results of assessments conducted prior to the care plan visit are allowed in care planning documentation provided they remain valid or are updated with any changes at the time of care planning. Palmetto GBA believes that the needed assessments and visits pursuant to the written comprehensive

treatment plan should all have occurred within a time period of $\approx$ 3 months in order to assure relevance of all such assessments to the treatment plan.

Many of the required assessment elements can be completed by appropriately trained members of the clinical team working with the eligible provider. Assessments that require the direct participation of a knowledgeable care partner or caregiver, such as a structured assessment of the patient's functioning at home or a caregiver stress measure, may be completed prior to the clinical visit and provided to the clinician for inclusion in care planning. Care planning visits can be conducted in the office or other outpatient setting, home, domiciliary or rest home settings, and via telehealth.

Though not required by CPT 99483, the time, complexity and expanse of required elements for this service may lend itself to a standardized care plan template. Such a template can be customized to the provider or health care system in order to simplify communication and tracking of patient care and outcomes over time. However, a template must be easily addressed and edited. Use of a templated document must provide for narrative that is unique and specific to the impacted beneficiary. No specific form or template is required for the written care plan as part of CPT 99483.

The written plan must be discussed with and given to the patient and/or family or caregiver; this face-to-face conversation must be documented in the clinical note for all encounters reported using 99483. The care plan must be filed in the patient's medical record where it can be easily retrieved and updated. Sharing the plan with other providers caring for the patient, including clinicians, care managers, caseworkers, and others who assist the patient and caregiver, both within and outside the primary care environment is highly recommended as it will help ensure continuity and coordination of care. When such sharing requires explicit consent of the patient, family, caregiver or legally designated decision-maker, that permission should be sought and documented.

Care plans should be revised at intervals and whenever there is a change in the patient's clinical or caregiving status. Palmetto GBA may audit the frequency of use for CPT 99483. Revisions of a care plan that do not include all the service elements of 99483 could be reported via other E/M codes such as chronic care management or non-face-to-face consultation codes.

A single physician or other qualified health care professional should not report 99483 more than once every 180 days.

99483 services are permanently covered via telehealth. Use CPT code 99483 to bill for both in-person and telehealth services. Although furnished via telehealth, all the required service elements for 99483 must still be present. Proper history acquisition from a corroborating or independent source must still occur.

Diagnosis Coding:

The condition(s) for which the patient receives Cognitive Assessment and Care Planning should be coded per the International Classification of Diseases, Tenth Revision, Clinical Modification (ICD 10-CM). When part of a Medical Wellness Visit, the code should report an administrative examination or a well exam diagnosis.

CPT Coding:

Some of the service elements under 99483 overlap with services under other E/M codes, advance care planning services, and certain psychological or psychiatric service codes per CPT coding directives and/or CMS guidance. As a result, the following CPT codes cannot be reported together with 99483 on the same date of service:

90785

90791

90792

96127

96146

96160-96161

99605-99607

99202-99215

99242

99243

99244

99245

99341

99342

99344

99345

99347

99348

99349

99350

99366-99368

99497

99498

Coding Information

CPT/HCPCS Codes

Group 1 (1 Code)

Group 1 Paragraph

N/A

Group 1 Codes

Code	Description
99483	ASSESSMENT OF AND CARE PLANNING FOR A PATIENT WITH COGNITIVE IMPAIRMENT, REQUIRING AN INDEPENDENT HISTORIAN, IN THE OFFICE OR OTHER OUTPATIENT, HOME OR DOMICILIARY OR REST HOME, WITH ALL OF THE FOLLOWING REQUIRED ELEMENTS: COGNITION-FOCUSED EVALUATION INCLUDING A PERTINENT HISTORY AND EXAMINATION, MEDICAL DECISION MAKING OF MODERATE OR HIGH COMPLEXITY, FUNCTIONAL ASSESSMENT (EG, BASIC AND INSTRUMENTAL ACTIVITIES OF DAILY LIVING), INCLUDING DECISION-MAKING CAPACITY, USE OF STANDARDIZED INSTRUMENTS FOR STAGING OF DEMENTIA (EG, FUNCTIONAL ASSESSMENT STAGING TEST [FAST], CLINICAL DEMENTIA RATING [CDR]), MEDICATION RECONCILIATION AND REVIEW FOR HIGH-RISK MEDICATIONS, EVALUATION FOR NEUROPSYCHIATRIC AND BEHAVIORAL SYMPTOMS, INCLUDING DEPRESSION, INCLUDING USE OF STANDARDIZED SCREENING INSTRUMENT(S), EVALUATION OF SAFETY (EG, HOME), INCLUDING MOTOR VEHICLE OPERATION, IDENTIFICATION OF CAREGIVER(S), CAREGIVER KNOWLEDGE, CAREGIVER NEEDS, SOCIAL

Code	Description
	SUPPORTS, AND THE WILLINGNESS OF CAREGIVER TO TAKE ON CAREGIVING TASKS, DEVELOPMENT, UPDATING OR REVISION, OR REVIEW OF AN ADVANCE CARE PLAN, CREATION OF A WRITTEN CARE PLAN, INCLUDING INITIAL PLANS TO ADDRESS ANY NEUROPSYCHIATRIC SYMPTOMS, NEURO-COGNITIVE SYMPTOMS, FUNCTIONAL LIMITATIONS, AND REFERRAL TO COMMUNITY RESOURCES AS NEEDED (EG, REHABILITATION SERVICES, ADULT DAY PROGRAMS, SUPPORT GROUPS) SHARED WITH THE PATIENT AND/OR CAREGIVER WITH INITIAL EDUCATION AND SUPPORT. TYPICALLY, 60 MINUTES OF TOTAL TIME IS SPENT ON THE DATE OF THE ENCOUNTER

Group 2 (36 Codes)

Group 2 Paragraph

The following CPT codes cannot be reported together with 99483 on the same date of service.

Group 2 Codes

Code	Description
90785	INTERACTIVE COMPLEXITY (LIST SEPARATELY IN ADDITION TO THE CODE FOR PRIMARY PROCEDURE)
90791	PSYCHIATRIC DIAGNOSTIC EVALUATION
90792	PSYCHIATRIC DIAGNOSTIC EVALUATION WITH MEDICAL SERVICES

Code	Description
96127	BRIEF EMOTIONAL/BEHAVIORAL ASSESSMENT (EG, DEPRESSION INVENTORY, ATTENTION-DEFICIT/HYPERACTIVITY DISORDER [ADHD] SCALE), WITH SCORING AND DOCUMENTATION, PER STANDARDIZED INSTRUMENT
96146	PSYCHOLOGICAL OR NEUROPSYCHOLOGICAL TEST ADMINISTRATION, WITH SINGLE AUTOMATED, STANDARDIZED INSTRUMENT VIA ELECTRONIC PLATFORM, WITH AUTOMATED RESULT ONLY
96160	ADMINISTRATION OF PATIENT-FOCUSED HEALTH RISK ASSESSMENT INSTRUMENT (EG, HEALTH HAZARD APPRAISAL) WITH SCORING AND DOCUMENTATION, PER STANDARDIZED INSTRUMENT
96161	ADMINISTRATION OF CAREGIVER-FOCUSED HEALTH RISK ASSESSMENT INSTRUMENT (EG, DEPRESSION INVENTORY) FOR THE BENEFIT OF THE PATIENT, WITH SCORING AND DOCUMENTATION, PER STANDARDIZED INSTRUMENT
99202	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND STRAIGHTFORWARD MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 15 MINUTES MUST BE MET OR EXCEEDED.
99203	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY

Code	Description
	APPROPRIATE HISTORY AND/OR EXAMINATION AND LOW LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 30 MINUTES MUST BE MET OR EXCEEDED.
99204	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND MODERATE LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 45 MINUTES MUST BE MET OR EXCEEDED.
99205	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND HIGH LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 60 MINUTES MUST BE MET OR EXCEEDED.
99211	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT THAT MAY NOT REQUIRE THE PRESENCE OF A PHYSICIAN OR OTHER QUALIFIED HEALTH CARE PROFESSIONAL

Code	Description
99212	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND STRAIGHTFORWARD MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 10 MINUTES MUST BE MET OR EXCEEDED.
99213	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND LOW LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 20 MINUTES MUST BE MET OR EXCEEDED.
99214	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND MODERATE LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 30 MINUTES MUST BE MET OR EXCEEDED.
99215	OFFICE OR OTHER OUTPATIENT VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND HIGH LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 40 MINUTES MUST BE MET OR EXCEEDED.

Code	Description
99242	OFFICE OR OTHER OUTPATIENT CONSULTATION FOR A NEW OR ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND STRAIGHTFORWARD MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 20 MINUTES MUST BE MET OR EXCEEDED.
99243	OFFICE OR OTHER OUTPATIENT CONSULTATION FOR A NEW OR ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND LOW LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 30 MINUTES MUST BE MET OR EXCEEDED.
99244	OFFICE OR OTHER OUTPATIENT CONSULTATION FOR A NEW OR ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND MODERATE LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 40 MINUTES MUST BE MET OR EXCEEDED.
99245	OFFICE OR OTHER OUTPATIENT CONSULTATION FOR A NEW OR ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND HIGH LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 55 MINUTES MUST BE MET OR EXCEEDED.

Code	Description
99341	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND STRAIGHTFORWARD MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 15 MINUTES MUST BE MET OR EXCEEDED.
99342	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND LOW LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 30 MINUTES MUST BE MET OR EXCEEDED.
99344	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND MODERATE LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 60 MINUTES MUST BE MET OR EXCEEDED.
99345	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF A NEW PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND HIGH LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 75 MINUTES MUST BE MET OR EXCEEDED.

Code	Description
99347	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND STRAIGHTFORWARD MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 20 MINUTES MUST BE MET OR EXCEEDED.
99348	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND LOW LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 30 MINUTES MUST BE MET OR EXCEEDED.
99349	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND MODERATE LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 40 MINUTES MUST BE MET OR EXCEEDED.
99350	HOME OR RESIDENCE VISIT FOR THE EVALUATION AND MANAGEMENT OF AN ESTABLISHED PATIENT, WHICH REQUIRES A MEDICALLY APPROPRIATE HISTORY AND/OR EXAMINATION AND HIGH LEVEL OF MEDICAL DECISION MAKING. WHEN USING TOTAL TIME ON THE DATE OF THE ENCOUNTER FOR CODE SELECTION, 60 MINUTES MUST BE MET OR EXCEEDED.

Code	Description
99366	MEDICAL TEAM CONFERENCE WITH INTERDISCIPLINARY TEAM OF HEALTH CARE PROFESSIONALS, FACE-TO-FACE WITH PATIENT AND/OR FAMILY, 30 MINUTES OR MORE, PARTICIPATION BY NONPHYSICIAN QUALIFIED HEALTH CARE PROFESSIONAL
99367	MEDICAL TEAM CONFERENCE WITH INTERDISCIPLINARY TEAM OF HEALTH CARE PROFESSIONALS, PATIENT AND/OR FAMILY NOT PRESENT, 30 MINUTES OR MORE; PARTICIPATION BY PHYSICIAN
99368	MEDICAL TEAM CONFERENCE WITH INTERDISCIPLINARY TEAM OF HEALTH CARE PROFESSIONALS, PATIENT AND/OR FAMILY NOT PRESENT, 30 MINUTES OR MORE; PARTICIPATION BY NONPHYSICIAN QUALIFIED HEALTH CARE PROFESSIONAL
99497	ADVANCE CARE PLANNING INCLUDING THE EXPLANATION AND DISCUSSION OF ADVANCE DIRECTIVES SUCH AS STANDARD FORMS (WITH COMPLETION OF SUCH FORMS, WHEN PERFORMED), BY THE PHYSICIAN OR OTHER QUALIFIED HEALTH CARE PROFESSIONAL; FIRST 30 MINUTES, FACE-TO-FACE WITH THE PATIENT, FAMILY MEMBER(S), AND/OR SURROGATE

Code	Description
99498	ADVANCE CARE PLANNING INCLUDING THE EXPLANATION AND DISCUSSION OF ADVANCE DIRECTIVES SUCH AS STANDARD FORMS (WITH COMPLETION OF SUCH FORMS, WHEN PERFORMED), BY THE PHYSICIAN OR OTHER QUALIFIED HEALTH CARE PROFESSIONAL; EACH ADDITIONAL 30 MINUTES (LIST SEPARATELY IN ADDITION TO CODE FOR PRIMARY PROCEDURE)
99605	MEDICATION THERAPY MANAGEMENT SERVICE(S) PROVIDED BY A PHARMACIST, INDIVIDUAL, FACE-TO-FACE WITH PATIENT, WITH ASSESSMENT AND INTERVENTION IF PROVIDED; INITIAL 15 MINUTES, NEW PATIENT
99606	MEDICATION THERAPY MANAGEMENT SERVICE(S) PROVIDED BY A PHARMACIST, INDIVIDUAL, FACE-TO-FACE WITH PATIENT, WITH ASSESSMENT AND INTERVENTION IF PROVIDED; INITIAL 15 MINUTES, ESTABLISHED PATIENT
99607	MEDICATION THERAPY MANAGEMENT SERVICE(S) PROVIDED BY A PHARMACIST, INDIVIDUAL, FACE-TO-FACE WITH PATIENT, WITH ASSESSMENT AND INTERVENTION IF PROVIDED; EACH ADDITIONAL 15 MINUTES (LIST SEPARATELY IN ADDITION TO CODE FOR PRIMARY SERVICE)

CPT/HCPCS Modifiers

Group 1 (1 Code)

Group 1 Paragraph

The following modifier would need to be appropriately utilized if the AWV and Cognitive Assessment and Care plan services are done at the same visit and would only be appropriate for a truly separate and fully complete provision of the 99483 service above and beyond all the AWV components.

Group 1 Codes

Code	Description
25	SIGNIFICANT, SEPARATELY IDENTIFIABLE EVALUATION AND MANAGEMENT SERVICE BY THE SAME PHYSICIAN ON THE SAME DAY OF THE PROCEDURE OR OTHER SERVICE: THE PHYSICIAN MAY NEED TO INDICATE THAT ON THE DAY A PROCEDURE OR SERVICE IDENTIFIED BY A CPTCODE WAS PERFORMED, THE PATIENT'S CONDITION REQUIRED A SIGNIFICANT, SEPARATELY IDENTIFIABLE E/M SERVICE ABOVE AND BEYOND THE OTHER SERVICE PROVIDED OR BEYOND THE USUAL PREOPERATIVE AND POSTOPERATIVE CARE ASSOCIATED WITH THE PROCEDURE THAT WAS PERFORMED. THE E/M SERVICE MAY BE PROMPTED BY THE SYMPTOM OR CONDITION FOR WHICH THE PROCEDURE AND/OR SERVICE WAS PROVIDED. AS SUCH, DIFFERENT DIAGNOSES ARE NOT REQUIRED FOR REPORTING OF THE E/M SERVICES ON THE SAME DATE. THIS CIRCUMSTANCE MAY BE REPORTED BY ADDING THE MODIFIER -25 TO THE APPROPRIATE LEVEL OF E/M SERVICE, OR THE SEPARATE FIVE DIGIT MODIFIER 09925 MAY BE USED. NOTE: THIS MODIFIER IS NOT USED TO REPORT AN E/M SERVICE THAT RESULTED IN A DECISION TO PERFORM SURGERY. SEE MODIFIER -57.

ICD-10-CM Codes that Support Medical Necessity

Group 1 (111 Codes)

Group 1 Paragraph

N/A

Group 1 Codes

Code	Description
F01.50	Vascular dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F01.A0	Vascular dementia, mild, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F01.A11	Vascular dementia, mild, with agitation
F01.A18	Vascular dementia, mild, with other behavioral disturbance
F01.A2	Vascular dementia, mild, with psychotic disturbance
F01.A3	Vascular dementia, mild, with mood disturbance

Code	Description
F01.A4	Vascular dementia, mild, with anxiety
F01.B0	Vascular dementia, moderate, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F01.B11	Vascular dementia, moderate, with agitation
F01.B18	Vascular dementia, moderate, with other behavioral disturbance
F01.B2	Vascular dementia, moderate, with psychotic disturbance
F01.B3	Vascular dementia, moderate, with mood disturbance
F01.B4	Vascular dementia, moderate, with anxiety
F01.C0	Vascular dementia, severe, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F01.C11	Vascular dementia, severe, with agitation
F01.C18	Vascular dementia, severe, with other behavioral disturbance

Code	Description
F01.C2	Vascular dementia, severe, with psychotic disturbance
F01.C3	Vascular dementia, severe, with mood disturbance
F01.C4	Vascular dementia, severe, with anxiety
F02.80	Dementia in other diseases classified elsewhere, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F02.A0	Dementia in other diseases classified elsewhere, mild, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F02.A11	Dementia in other diseases classified elsewhere, mild, with agitation
F02.A18	Dementia in other diseases classified elsewhere, mild, with other behavioral disturbance
F02.A2	Dementia in other diseases classified elsewhere, mild, with psychotic disturbance
F02.A3	Dementia in other diseases classified elsewhere, mild, with mood disturbance
F02.A4	Dementia in other diseases classified elsewhere, mild, with anxiety

Code	Description
F02.B0	Dementia in other diseases classified elsewhere, moderate, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F02.B11	Dementia in other diseases classified elsewhere, moderate, with agitation
F02.B18	Dementia in other diseases classified elsewhere, moderate, with other behavioral disturbance
F02.B2	Dementia in other diseases classified elsewhere, moderate, with psychotic disturbance
F02.B3	Dementia in other diseases classified elsewhere, moderate, with mood disturbance
F02.B4	Dementia in other diseases classified elsewhere, moderate, with anxiety
F02.C0	Dementia in other diseases classified elsewhere, severe, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F02.C11	Dementia in other diseases classified elsewhere, severe, with agitation
F02.C18	Dementia in other diseases classified elsewhere, severe, with other behavioral disturbance

Code	Description
F02.C2	Dementia in other diseases classified elsewhere, severe, with psychotic disturbance
F02.C3	Dementia in other diseases classified elsewhere, severe, with mood disturbance
F02.C4	Dementia in other diseases classified elsewhere, severe, with anxiety
F03.90	Unspecified dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F03.911	Unspecified dementia, unspecified severity, with agitation
F03.918	Unspecified dementia, unspecified severity, with other behavioral disturbance
F03.92	Unspecified dementia, unspecified severity, with psychotic disturbance
F03.93	Unspecified dementia, unspecified severity, with mood disturbance
F03.94	Unspecified dementia, unspecified severity, with anxiety
F03.A0	Unspecified dementia, mild, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety

Code	Description
F03.A11	Unspecified dementia, mild, with agitation
F03.A18	Unspecified dementia, mild, with other behavioral disturbance
F03.A2	Unspecified dementia, mild, with psychotic disturbance
F03.A3	Unspecified dementia, mild, with mood disturbance
F03.A4	Unspecified dementia, mild, with anxiety
F03.B0	Unspecified dementia, moderate, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F03.B11	Unspecified dementia, moderate, with agitation
F03.B18	Unspecified dementia, moderate, with other behavioral disturbance
F03.B2	Unspecified dementia, moderate, with psychotic disturbance
F03.B3	Unspecified dementia, moderate, with mood disturbance
F03.B4	Unspecified dementia, moderate, with anxiety

Code	Description
F03.C0	Unspecified dementia, severe, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
F03.C11	Unspecified dementia, severe, with agitation
F03.C18	Unspecified dementia, severe, with other behavioral disturbance
F03.C2	Unspecified dementia, severe, with psychotic disturbance
F03.C3	Unspecified dementia, severe, with mood disturbance
F03.C4	Unspecified dementia, severe, with anxiety
F06.70	Mild neurocognitive disorder due to known physiological condition without behavioral disturbance
F06.71	Mild neurocognitive disorder due to known physiological condition with behavioral disturbance
F10.27	Alcohol dependence with alcohol-induced persisting dementia
F10.97	Alcohol use, unspecified with alcohol-induced persisting dementia

Code	Description
F13.27	Sedative, hypnotic or anxiolytic dependence with sedative, hypnotic or anxiolytic-induced persisting dementia
F13.288	Sedative, hypnotic or anxiolytic dependence with other sedative, hypnotic or anxiolytic-induced disorder
F13.97	Sedative, hypnotic or anxiolytic use, unspecified with sedative, hypnotic or anxiolytic-induced persisting dementia
F18.17	Inhalant abuse with inhalant-induced dementia
F18.188	Inhalant abuse with other inhalant-induced disorder
F18.27	Inhalant dependence with inhalant-induced dementia
F18.288	Inhalant dependence with other inhalant-induced disorder
F18.97	Inhalant use, unspecified with inhalant-induced persisting dementia
F18.988	Inhalant use, unspecified with other inhalant-induced disorder

Code	Description
F19.17	Other psychoactive substance abuse with psychoactive substance-induced persisting dementia
F19.188	Other psychoactive substance abuse with other psychoactive substance-induced disorder
F19.27	Other psychoactive substance dependence with psychoactive substance-induced persisting dementia
F19.288	Other psychoactive substance dependence with other psychoactive substance-induced disorder
F19.97	Other psychoactive substance use, unspecified with psychoactive substance-induced persisting dementia
F19.988	Other psychoactive substance use, unspecified with other psychoactive substance-induced disorder
G10	Huntington's disease
G30.0	Alzheimer's disease with early onset
G30.1	Alzheimer's disease with late onset

Code	Description
G30.8	Other Alzheimer's disease
G31.01	Pick's disease
G31.09	Other frontotemporal neurocognitive disorder
G31.2	Degeneration of nervous system due to alcohol
G31.80	Leukodystrophy, unspecified
G31.83	Neurocognitive disorder with Lewy bodies
G31.84	Mild cognitive impairment of uncertain or unknown etiology
G31.85	Corticobasal degeneration
G31.86	Alexander disease
G31.87	Primary progressive apraxia of speech
I69.010	Attention and concentration deficit following nontraumatic subarachnoid hemorrhage

Code	Description
I69.011	Memory deficit following nontraumatic subarachnoid hemorrhage
I69.014	Frontal lobe and executive function deficit following nontraumatic subarachnoid hemorrhage
I69.015	Cognitive social or emotional deficit following nontraumatic subarachnoid hemorrhage
I69.210	Attention and concentration deficit following other nontraumatic intracranial hemorrhage
I69.211	Memory deficit following other nontraumatic intracranial hemorrhage
I69.214	Frontal lobe and executive function deficit following other nontraumatic intracranial hemorrhage
I69.215	Cognitive social or emotional deficit following other nontraumatic intracranial hemorrhage
I69.310	Attention and concentration deficit following cerebral infarction
I69.311	Memory deficit following cerebral infarction

Code	Description
I69.314	Frontal lobe and executive function deficit following cerebral infarction
I69.315	Cognitive social or emotional deficit following cerebral infarction
I69.810	Attention and concentration deficit following other cerebrovascular disease
I69.811	Memory deficit following other cerebrovascular disease
I69.814	Frontal lobe and executive function deficit following other cerebrovascular disease
I69.815	Cognitive social or emotional deficit following other cerebrovascular disease
R41.81	Age-related cognitive decline

ICD-10-CM Codes that DO NOT Support Medical Necessity

N/A

ICD-10-PCS Codes

N/A

Additional ICD-10 Information

N/A

Bill Type Codes

Contractors may specify Bill Types to help providers identify those Bill Types typically used to report this service. Absence of a Bill Type does not guarantee that the article does not apply to that Bill Type. Complete absence of all Bill Types indicates that coverage is not influenced by Bill Type and the article should be assumed to apply equally to all claims.

N/A

Revenue Codes

Contractors may specify Revenue Codes to help providers identify those Revenue Codes typically used to report this service. In most instances Revenue Codes are purely advisory. Unless specified in the article, services reported under other Revenue Codes are equally subject to this coverage determination. Complete absence of all Revenue Codes indicates that coverage is not influenced by Revenue Code and the article should be assumed to apply equally to all Revenue Codes.

N/A

Other Coding Information

N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation
01/15/2026	R8	Under <i>CPT/HCPCS Codes Group 1: Codes</i> the description was revised for 99483. This revision is due to the 2026 Annual/Q1 CPT/HCPCS Code Update and is retroactive effective for dates of service on or after 1/1/26.
01/15/2026	R7	Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Codes</i> added F03.92, F03.93, F03.94, F03.A0, F03.A11, F03.A18, F03.A2, F03.A3, F03.A4, F03.B0, F03.B11, F03.B18, F03.B2, F03.B3, F03.B4, F03.C0, F03.C11, F03.C18, F03.C2, F03.C3 and F03.C4.
12/15/2025	R6	Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Codes</i> added F03.90, F03.911 and F03.918.

Revision History Date	Revision History Number	Revision History Explanation
10/01/2025	R5	Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Codes</i> added G31.87. This revision is due to the Annual ICD-10-CM Update and will become effective on 10/1/25.
01/01/2024	R4	Under <i>CPT/HCPCS Codes Group 2: Codes</i> the description was revised for 99202, 99203, 99204, 99205, 99212, 99213, 99214 and 99215. This revision is due to the 2024 Annual/Q1 CPT/HCPCS Code Update and is retroactive effective for dates of service on or after 1/1/24.
10/01/2023	R3	Under <i>ICD-10 Codes that Support Medical Necessity Group 1: Codes</i> added G31.80 and G31.86. This revision is due to the Annual ICD-10-CM Update and will become effective on 10/1/23.
01/01/2023	R2	Under <i>Article Text</i> subheading CPT Coding deleted the CPT codes 99241, 99324-99337 and 99343. Under <i>CPT/HCPCS Codes Group 1: Codes</i> the description was revised for 99483. Under <i>CPT/HCPCS Codes Group 2: Codes</i> deleted 99241, 99324, 99325, 99326, 99327, 99328, 99334, 99335, 99336, 99337, 99343 and the description was revised for 99242, 99243, 99244, 99245, 99341, 99342, 99344, 99345, 99347, 99348, 99349, 99350. This revision is due to the

Revision History Date	Revision History Number	Revision History Explanation
		2023 Annual/Q1 CPT/HCPCS Code Update and is retroactive effective for dates of service on or after 1/1/23.
10/01/2022	R1	Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Codes</i> deleted F03.90. This deletion is effective 8/27/22. Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Codes</i> deleted F01.51, F02.81, F03.91 and added F01.A0, F01.A11, F01.A18, F01.A2, F01.A3, F01.A4, F01.B0, F01.B11, F01.B18, F01.B2, F01.B3, F01.B4, F01.C0, F01.C11, F01.C18, F01.C2, F01.C3, F01.C4, F02.A0, F02.A11, F02.A18, F02.A2, F02.A3, F02.A4, F02.B0, F02.B11, F02.B18, F02.B2, F02.B3, F02.B4, F02.C0, F02.C11, F02.C18, F02.C2, F02.C3, F02.C4, F06.70, F06.71. Code descriptions were revised for F01.50, F02.80, G31.09, G31.83, and G31.84. This revision is due to the Annual ICD-10-CM Update and will become effective on 10/1/22.

Associated Documents

Related Local Coverage Documents
LCDs

L39266 - Cognitive Assessment and Care Plan Service [↗](#)

Related National Coverage Documents

NCDs

N/A

Statutory Requirements URLs

N/A

Rules and Regulations URLs

N/A

CMS Manual Explanations URLs

N/A

Other URLs

N/A

Public Versions

Updated On	Effective Dates	Status	
02/04/2026	01/15/2026 - N/A	Currently in Effect	You are here
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08/26/2025	10/01/2025 - 12/14/2025	Superseded	View
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Keywords

- Cognitive Assessment
- Care Plan Service